Start with a conversation, not a conclusion

Memory and thinking concerns can have different explanations. A health professional can assess the concern and discuss next steps. This worksheet cannot tell you whether someone has dementia.

Where possible, invite the person to describe what they have noticed and what they want help understanding. Ask whether they would like another person involved. Do not use the worksheet to test, confront, or correct them.

> Sudden confusion or trouble speaking, sudden one-sided weakness, new trouble seeing or walking, or a sudden severe unexplained headache can be stroke symptoms. Call 911 immediately; do not wait to complete this guide.

What I have noticed, in my own words:

What I would like help understanding:

National Institute on Aging — Memory Problems, Forgetfulness, and Aging
https://www.nia.nih.gov/health/memory-loss-and-forgetfulness/memory-problems-forgetfulness-and-aging
Centers for Disease Control and Prevention — Signs and Symptoms of Stroke
https://www.cdc.gov/stroke/signs-symptoms/index.html

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Describe a few specific observations

Brief examples can be easier to discuss than a broad label. Record what happened, roughly when it happened, and how it affected an ordinary activity. Separate direct observations from assumptions.

For example, a neutral note describes an event rather than calling someone “careless” or “confused all the time.” Include the person’s own explanation when they want it included.

Write only what is needed for the conversation. This is not a symptom score or a diagnostic checklist.

What happened, and approximately when?

Was this new or different from what is usual for the person?

How did it affect the activity or the person?

What did the person say about it?

Bring questions and practical information

Choose a few questions that matter most. Bring an up-to-date medication list, including nonprescription medicines and supplements, so the clinician can review it. Do not stop or change prescribed medication on your own.

Possible questions: “What could explain the changes?” “What assessment would be useful?” “What should we watch for?” “Who should we contact, and when?” “What support is available while we work through this?”

Tell the office in advance about communication or access needs. Ask the person whether they would like a support person at the appointment.

The person’s top question:

Another question we want to ask:

Information or documents we plan to bring:

National Institute on Aging — Cognitive Health and Older Adults
https://www.nia.nih.gov/health/brain-health/cognitive-health-and-older-adults

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Leave with a clear next step

Before the appointment ends, ask for an explanation of the plan in language you understand. A support person can help record it when the person agrees. Ask for written instructions or an accessible format where needed.

The website directory “Find Your Next Step” identifies established information and support services. These are independent organizations, not services delivered by The Maxine Project.

This worksheet is not a diagnosis, medical record, or substitute for the clinician’s instructions. Store it privately and share it only with appropriate permission.

What we understand from the appointment:

The next step, and who is responsible:

When to follow up—and whom to contact with questions:

Support the person would welcome:

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